START HERE

# One patient, from arrival to exit

Lotus Grove Demo Hospital | Fictional training case

- **Patient:** Demo Nisha Kapoor, age 45, female. UHID **DNK9031801**; stay **IP-2026-00007**; insured; bed Demo G-104; treating Doctor Dr Demo Anika Rao.
- **The story:** Nisha is admitted, receives a fictional consultation, blood count and medicine packs, then completes an ordinary insured discharge. Every screenshot is from the actual local app.
- **The six dashboards:** Admission registers/activates; Admin assigns people and releases beds; Nurse coordinates readiness and exit; Pharmacy reconciles medicine; Insurance records accepted responsibility; Billing settles money.
- **Money:** gross ₹3,000 less ₹100 medicine return = ₹2,900. Patient owes ₹900; insurer accepts ₹2,000. Deposit ₹2,000; patient refund ₹1,100.
- **This journey is already completed.** Find Nisha in Retained MVP episodes. To practise, create a new fictional patient with unique receipt/invoice references; do not enter Nisha's receipts again.




The real completed case: patient ID, admission number, bed and DISCHARGED status.

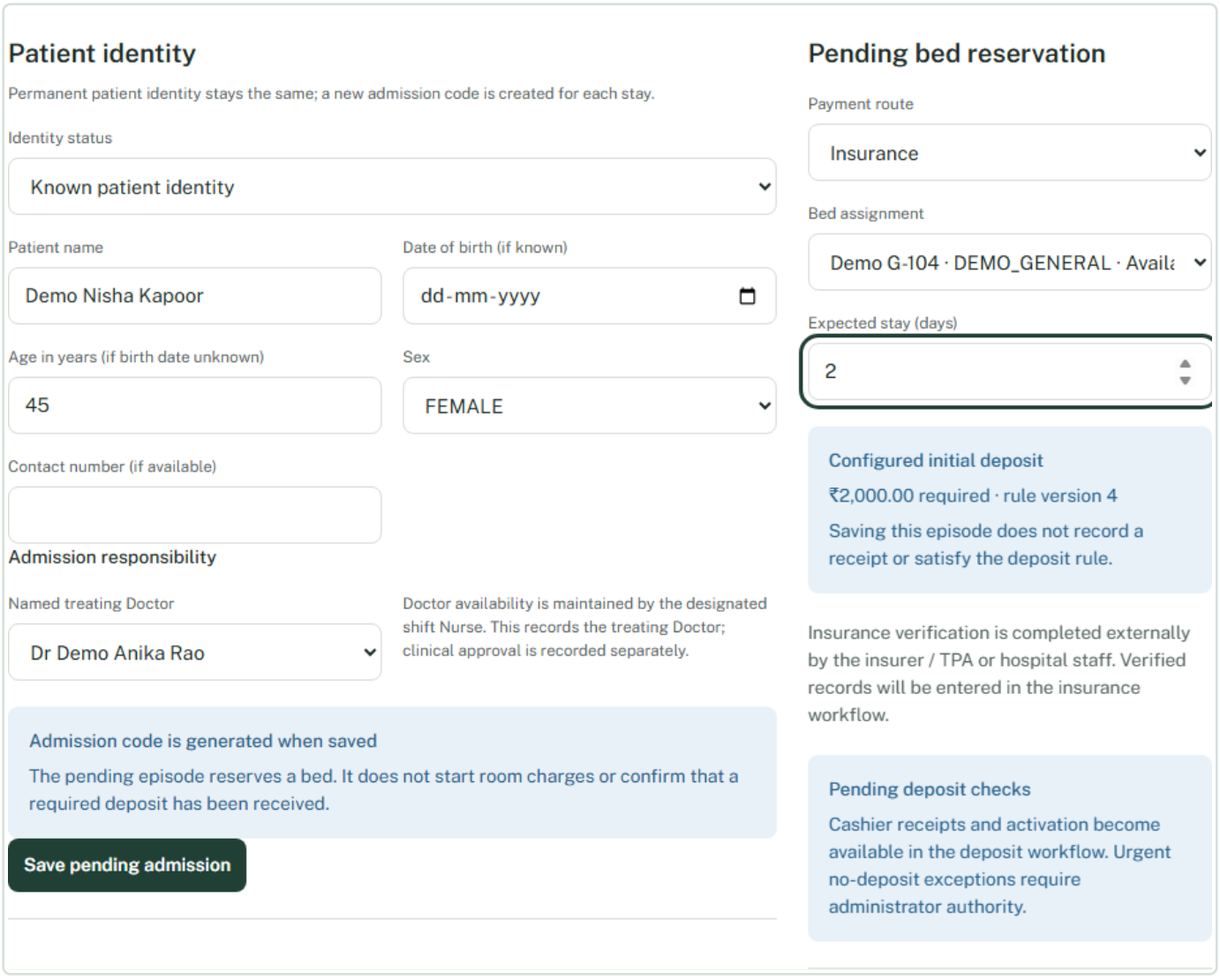
**Expected / verified:** Completed with two observed UI issues: Admission insurance handoff and the closed paper-status label. Workarounds and priorities are on pages 21-22.
**Next:** Follow the numbered pages; switch roles only when the owner changes.

ADMISSION

# Register and reserve a bed

Admissions > New admission

- **Search first** so an existing patient gets another stay rather than a second patient identity. For this new fictional case, enter Demo Nisha Kapoor, age 45 and FEMALE; leave unknown contact details blank.
- **Select** Dr Demo Anika Rao, available Demo G-104, Insurance and expected stay 2 days. The configured insured deposit is ₹2,000.
- **Click Save pending admission.** Note the generated UHID and IP number, then open the admission record. A reservation alone does not collect money or start room charges.




Register Nisha and reserve Demo G-104

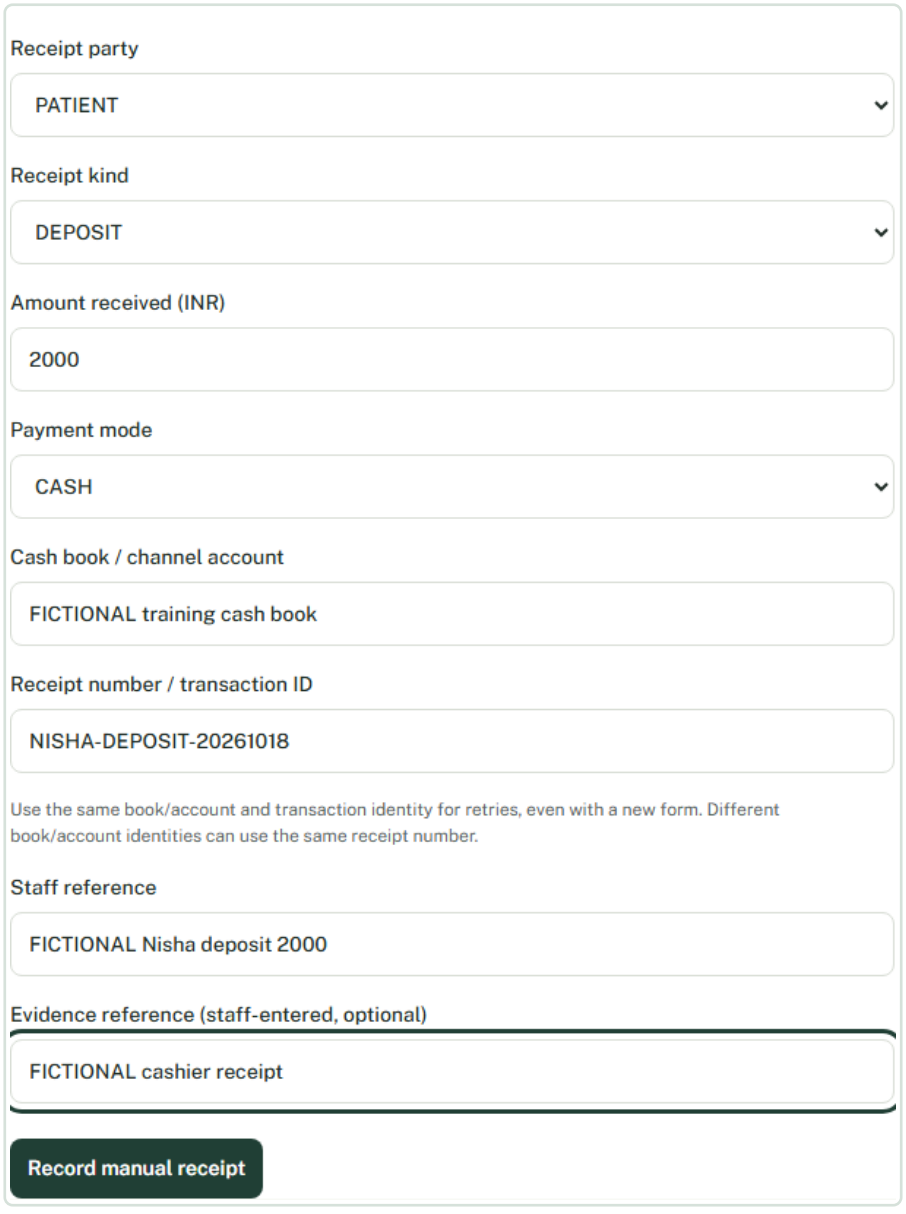
**Expected / verified:** PENDING admission saved and the bed reserved. Permanent patient identity and this stay's admission number are separate.
**Next:** Admission records the deposit.

ADMISSION

# Record the patient deposit

Nisha admission record > Patient deposit

- **Enter ₹2,000** as PATIENT / DEPOSIT. In this example the mode is CASH and the book is FICTIONAL training cash book.
- **Use a unique receipt reference:** NISHA-DEPOSIT-20261018, staff reference FICTIONAL Nisha deposit 2000, and a fictional evidence reference.
- **Click Record manual receipt.** This records a hospital-channel receipt; the app does not move or independently verify bank money. Do not add the same deposit again in Billing.




Record the fictional INR 2,000 deposit

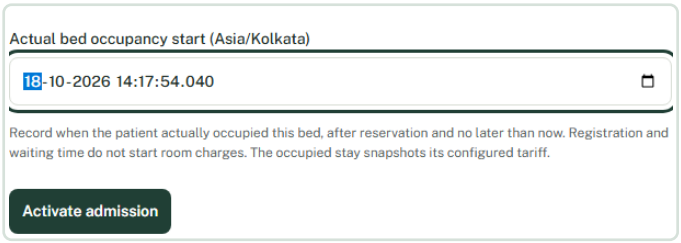
**Expected / verified:** The deposit check shows the initial ₹2,000 received. Arrival and activation still need a separate action.
**Next:** Admission records actual arrival.

ADMISSION → ADMIN

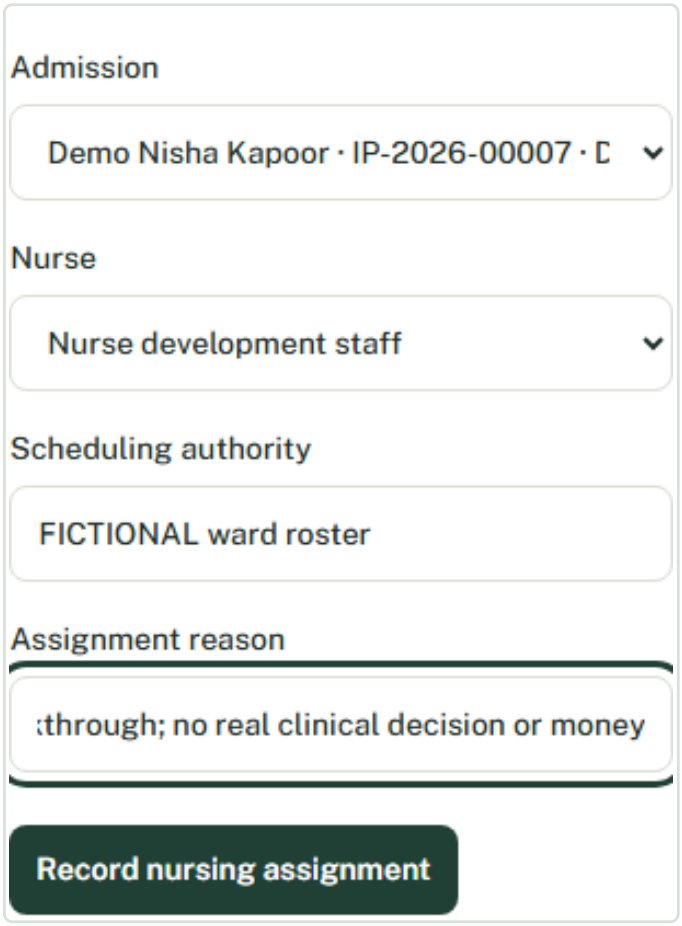
# Activate, then assign the Nurse

Actual arrival > Activate admission; Admin > Assign responsible Nurse

- **Admission:** enter the actual bed-occupancy start in Asia/Kolkata and click Activate admission. Use a time after reservation and no later than now.
- **Admin:** open the patient's coordination page and Assign responsible Nurse, which leads to Assignments. Select Nisha, the existing Nurse, scheduling authority and reason; click Record nursing assignment.
- **Why two actions?** Activation starts actual room use. The Nurse assignment separately establishes who can coordinate this patient's care and exit.




Record actual bed occupancy and activate




Admin assigns the responsible Nurse

**Expected / verified:** Nisha is ADMITTED; room charging starts at the configured ₹1,200 per started 24 hours. Nurse responsibility is recorded.
**Next:** The assigned Nurse starts a care shift.

NURSE

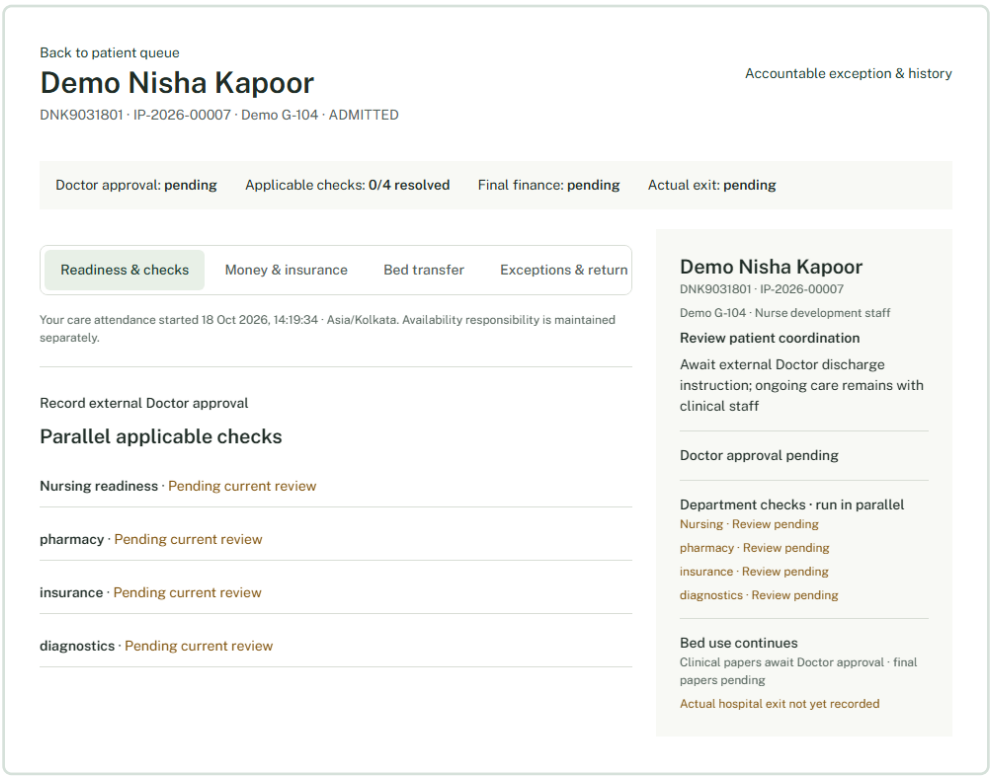
# Open the patient and start your shift

My patients > All patients > Nisha > Readiness & checks

- **Choose All patients** if Nisha is absent from Needs action. Open her patient coordination page; do not use another patient's shift.
- **Open Start assigned care shift,** record the actual shift start and incoming handover, then click Start care shift.
- **Read the status panel.** Doctor approval, applicable checks, finance and actual exit are separate. Starting a shift supplies attendance responsibility; it does not complete the discharge checks.




Nurse starts her assigned care shift




The patient coordination page and current next work.

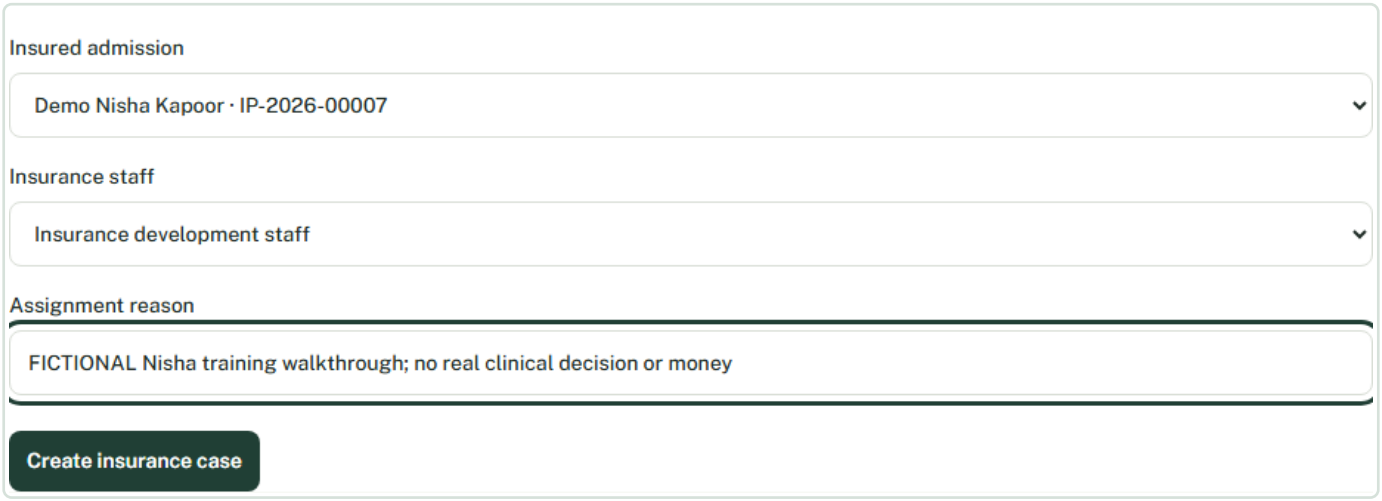
**Expected / verified:** The assigned Nurse now has an open care shift for later vacancy and exit actions.
**Next:** Pharmacy and Billing record the performed work.

ADMISSION

# Create and assign the insurance case

Admission record > Identity & intake > Create limited insurance case

- **Open Create and assign an insured case.** Select Nisha, choose the existing Insurance staff account, and enter the assignment reason. Click Create insurance case.
- **Hand off to the assigned Insurance user.** This links one case to Nisha's admission. Policy verification and payer decisions belong to the authorized Insurance review, not to Reception.
- **Observed caution:** the admission-linked Insurance detail shows 404 after case creation. The save succeeded. Return to Admissions and have the assigned Insurance user open Insurance cases; do not create another case.
- **In ordinary work, verify policy early.** This demonstration records the policy and final payer allocation on pages 14-15 after room use is finalized, so the example bill has stopped changing.




Create and assign Nisha's insured case

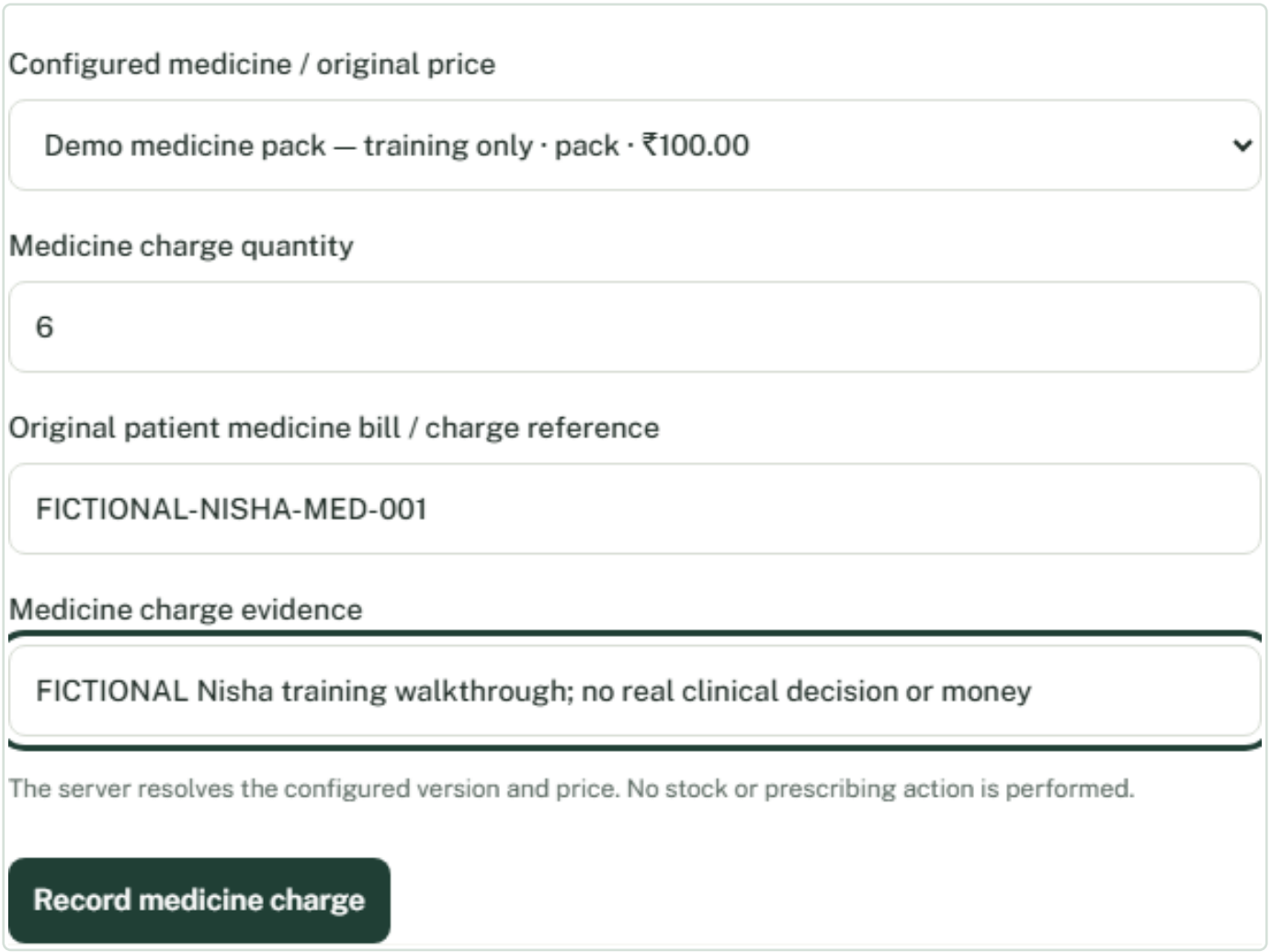
**Expected / verified:** Nisha's insured case is created and assigned; the detailed review is available under the authorized Insurance login.
**Next:** Pharmacy/Billing record services; Insurance can verify policy in parallel.

PHARMACY

# Record the patient medicine charge

Medicine bills & returns > Nisha > Medicine bills & returns tab

- **Open Record patient medicine charge.** Choose the configured Demo medicine pack at ₹100 per pack and enter quantity 6.
- **Record the original bill/source reference** FICTIONAL-NISHA-MED-001 and fictional supporting evidence. Click Record medicine charge.
- **The app applies the configured price:** 6 × ₹100 = ₹600. This is patient billing; this MVP screen does not prescribe or manage stock batches.




Charge six training medicine packs at INR 100 each

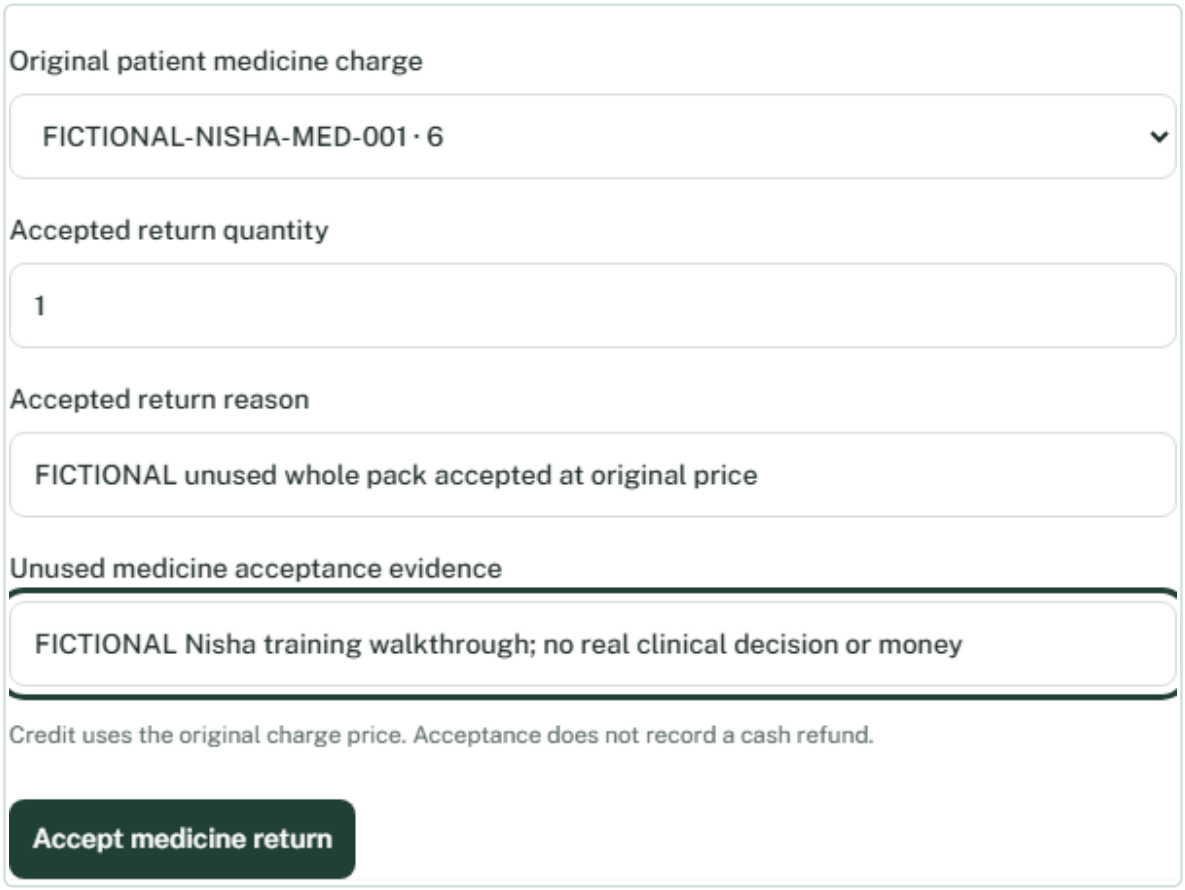
**Expected / verified:** The original ₹600 medicine charge is retained against this admission.
**Next:** Pharmacy records one accepted unused return.

PHARMACY

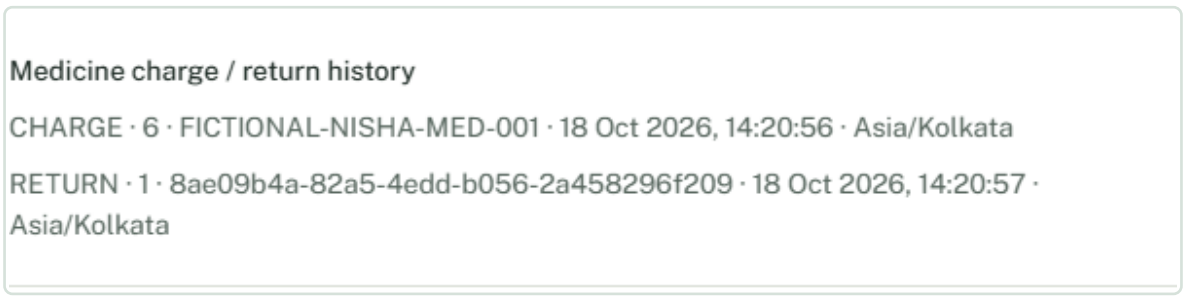
# Accept an unused medicine return

Same patient > Accept unused medicine return

- **Select the original medicine charge,** enter accepted return quantity 1, and record acceptance reason and evidence.
- **Click Accept medicine return.** The original pack price supplies a ₹100 credit; net medicine cost is now ₹500.
- **This is a bill credit.** It does not refund the patient's cash. Billing handles any actual excess-deposit refund later.




Accept one unused pack; bill credit is INR 100




Original charge and accepted return both remain visible

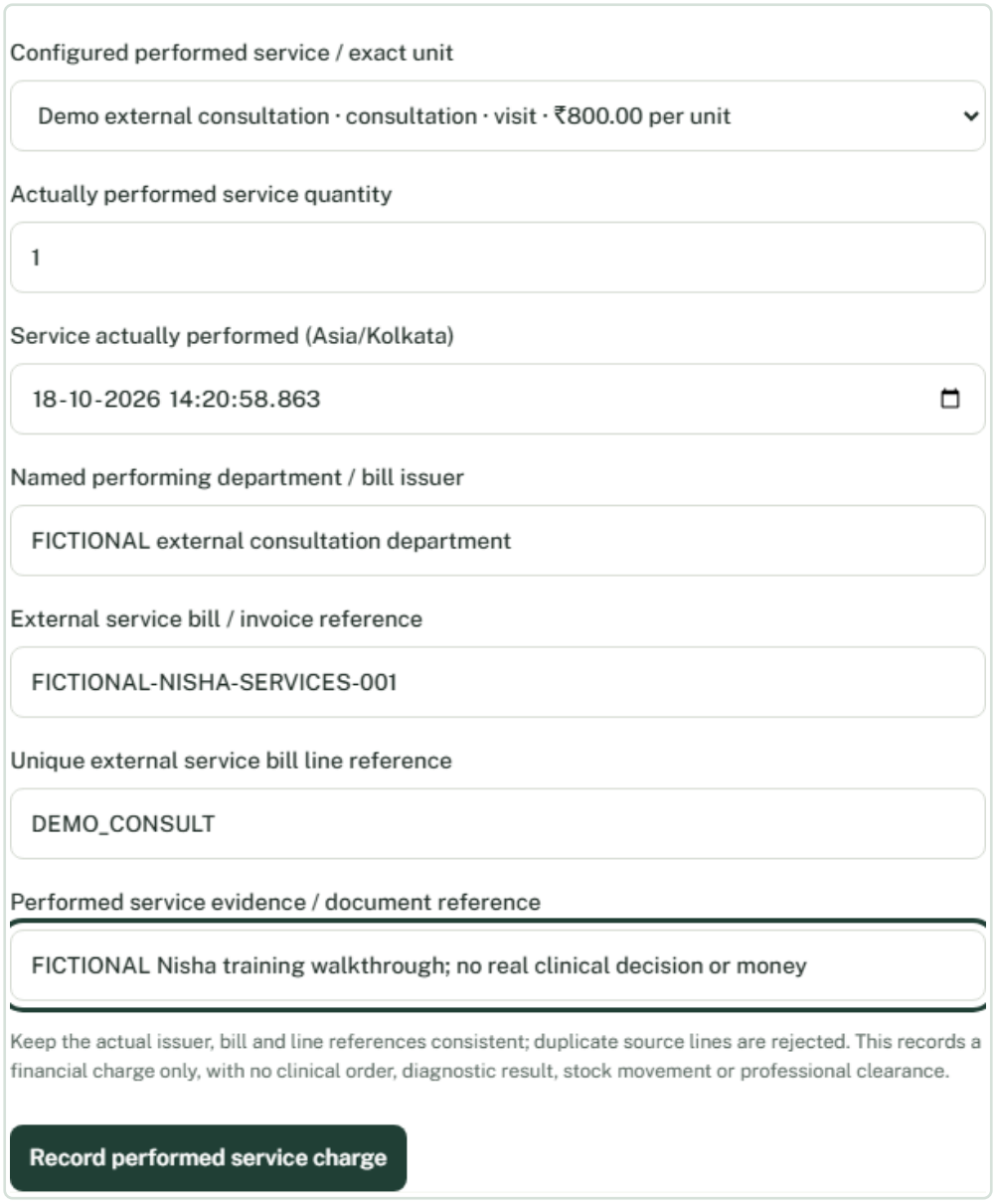
**Expected / verified:** Both the charge and accepted return remain visible in medicine history.
**Next:** Billing records externally performed services.

BILLING

# Record performed service charges

Payments & insurance > Nisha > Money & insurance

- **Open Record a performed service charge.** Choose Demo external consultation, quantity 1: configured price ₹800.
- **Record actual performed time, issuer, invoice and unique line reference.** Click Record performed service charge. Repeat for Demo blood count, quantity 1 at ₹400.
- **Use the same invoice but different line identities** for distinct services. These financial entries do not create a clinical order or a diagnostic result. Room use is reconciled at actual vacancy.




Record a performed consultation service at the configured price

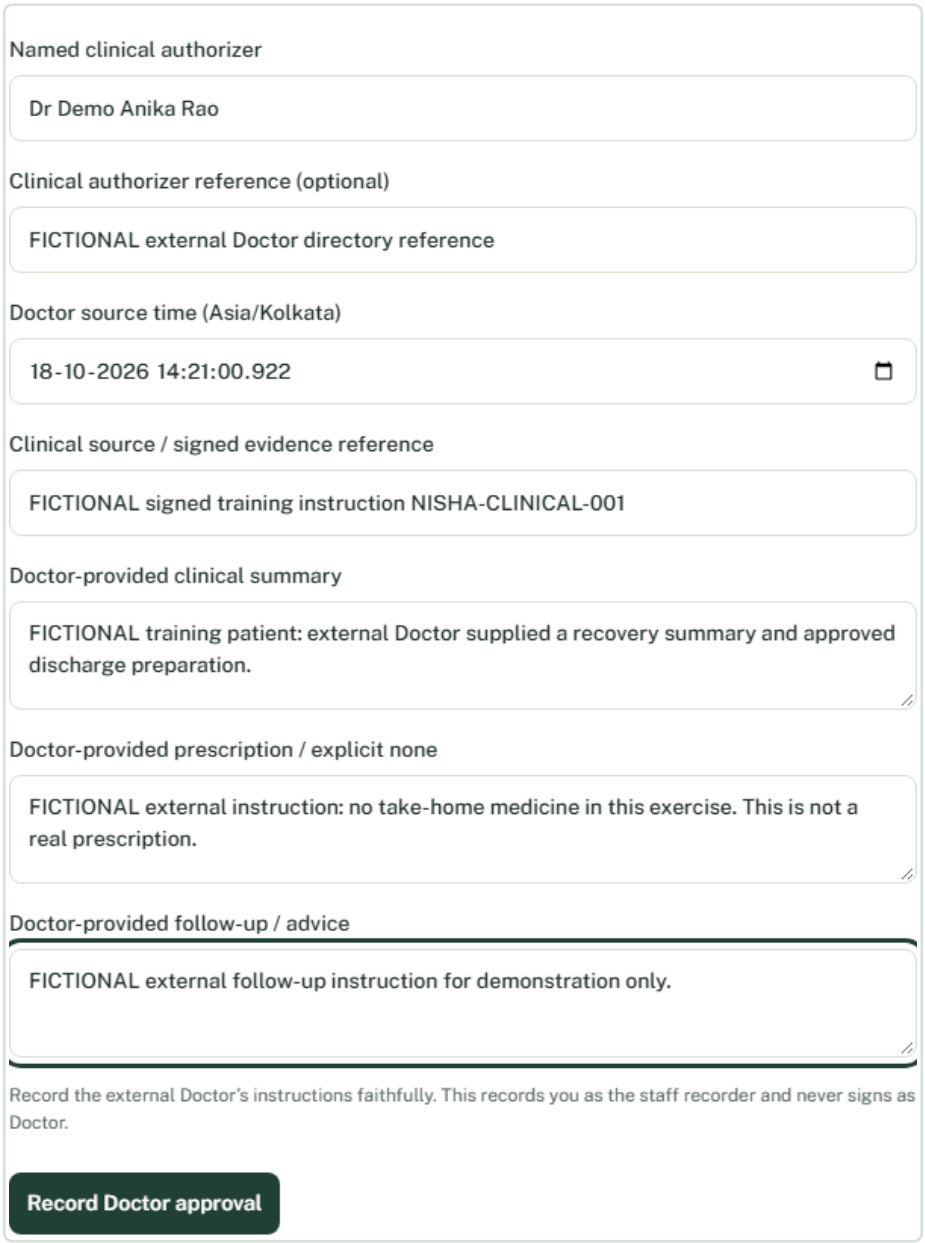
**Expected / verified:** Consultation ₹800 + blood count ₹400 + net medicines ₹500 are recorded. The later final room charge adds ₹1,200.
**Next:** The Doctor gives instructions externally; Nurse records the evidence.

NURSE

# Record the external Doctor instruction

Nisha > Readiness & checks > Record external Doctor approval

- **Enter the named authorizer:** Dr Demo Anika Rao, source time and the fictional signed-instruction evidence. The Nurse is recorded separately as the staff recorder.
- **Copy the Doctor-provided summary, prescription/explicit none and follow-up.** In this fictional exercise the instruction says no take-home medicine. Do not invent clinical content in real use.
- **Click Record Doctor approval.** Clinical papers can now be generated under Vacancy, papers & exit, even before final finance. We generated and reviewed that clinical version during this walk.




Nurse records the external Doctor's instruction and evidence

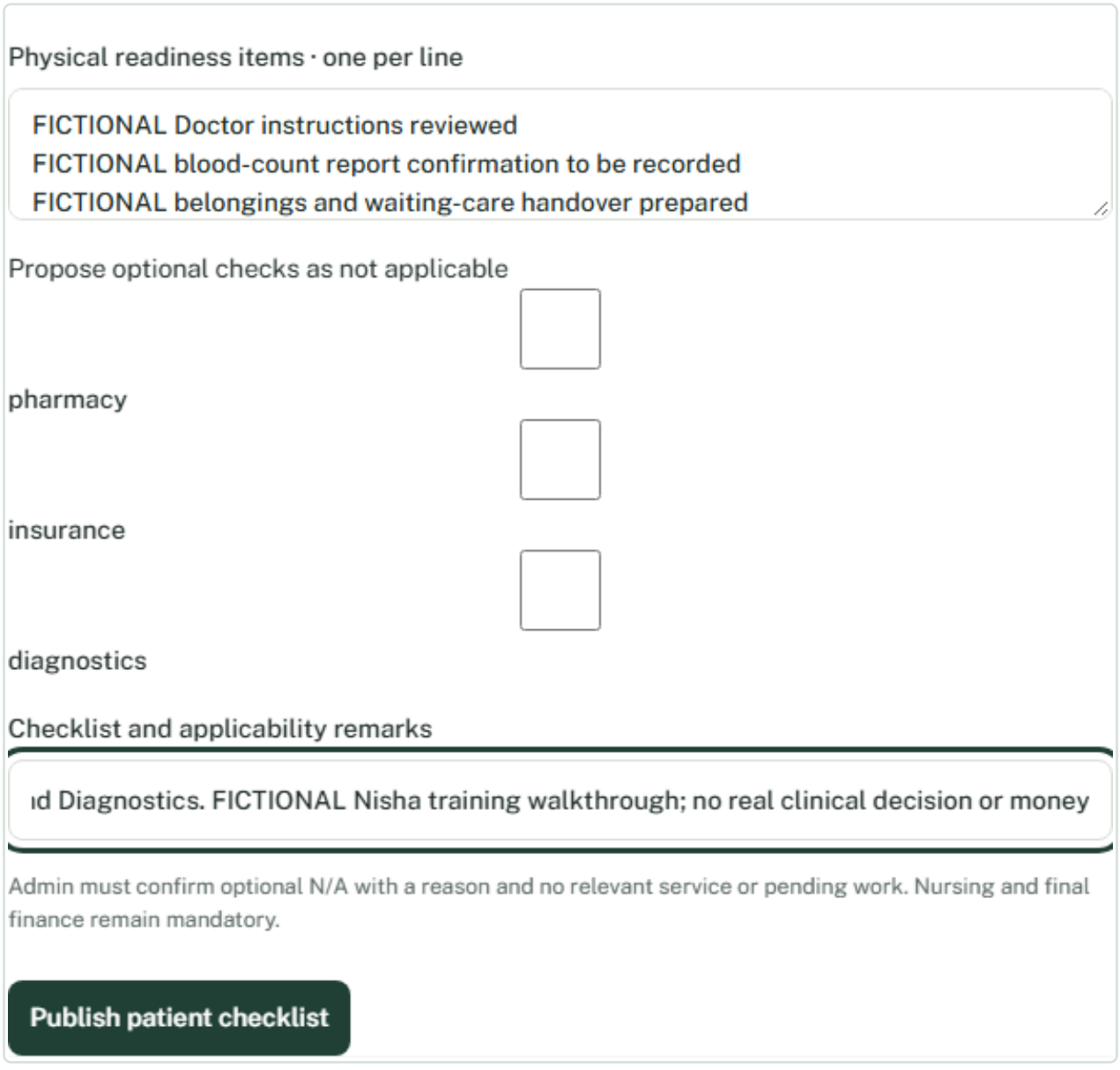
**Expected / verified:** External clinical authority is recorded without signing in or signing as the Doctor.
**Next:** Nurse publishes the current readiness checklist.

NURSE

# Publish the readiness checklist

Readiness & checks > Publish physical patient checklist

- **List the physical readiness items** one per line: instructions reviewed, diagnostic confirmation required, belongings and waiting-care handover prepared.
- **All four checks apply to Nisha:** Nursing, Pharmacy, Insurance and Diagnostics. No N/A is proposed for this insured patient with medicine and test charges.
- **Click Publish patient checklist.** Applicable checks can run in parallel. If no service applies in another case, Nurse proposes optional N/A and Admin confirms it with a reason.




Publish the checklist: all four department checks apply

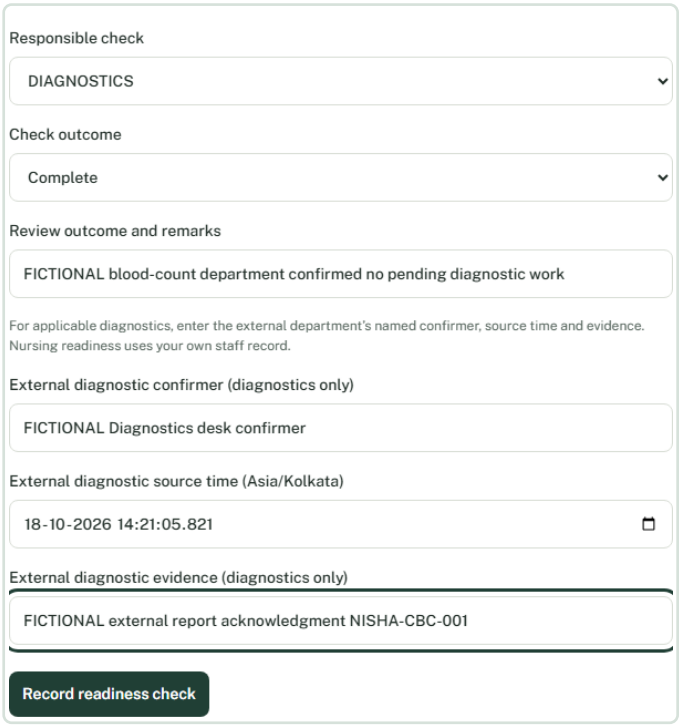
**Expected / verified:** A current physical checklist links the department checks to the recorded Doctor instruction.
**Next:** Nurse, Pharmacy and Insurance complete their own relevant checks.

NURSE + PHARMACY

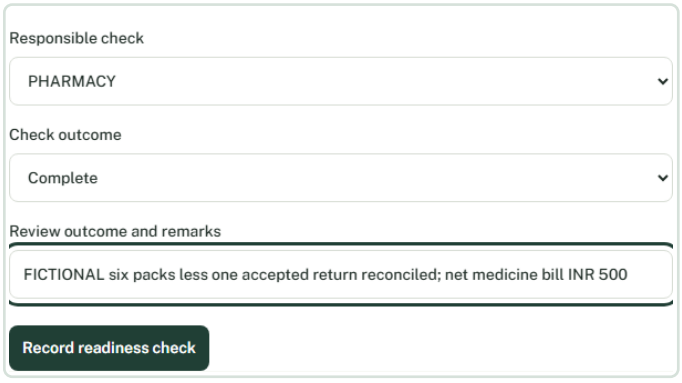
# Complete the relevant readiness checks

Readiness & checks > Record your readiness check

- **Nurse:** select NURSING, Complete, add the reviewed outcome and record the check.
- **Nurse:** select DIAGNOSTICS and enter the external department's named confirmer, source time and evidence before recording completion. A bill entry alone did not prove the result was confirmed.
- **Pharmacy:** use its own login, select PHARMACY and complete reconciliation of the six packs less one accepted return. Insurance will complete its check after the final payer allocation.




Record named external diagnostic confirmation




Pharmacy completes its own reconciliation check

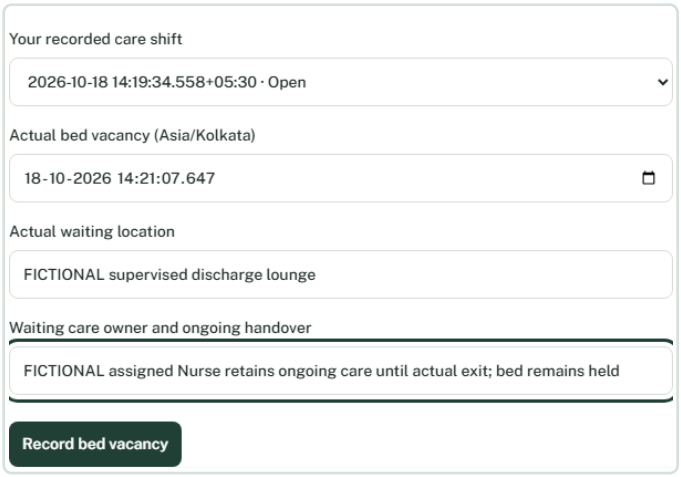
**Expected / verified:** Nursing, diagnostic confirmation and Pharmacy reconciliation are recorded by the responsible roles. Insurance is still pending.
**Next:** Nurse records the actual end of physical bed use.

NURSE

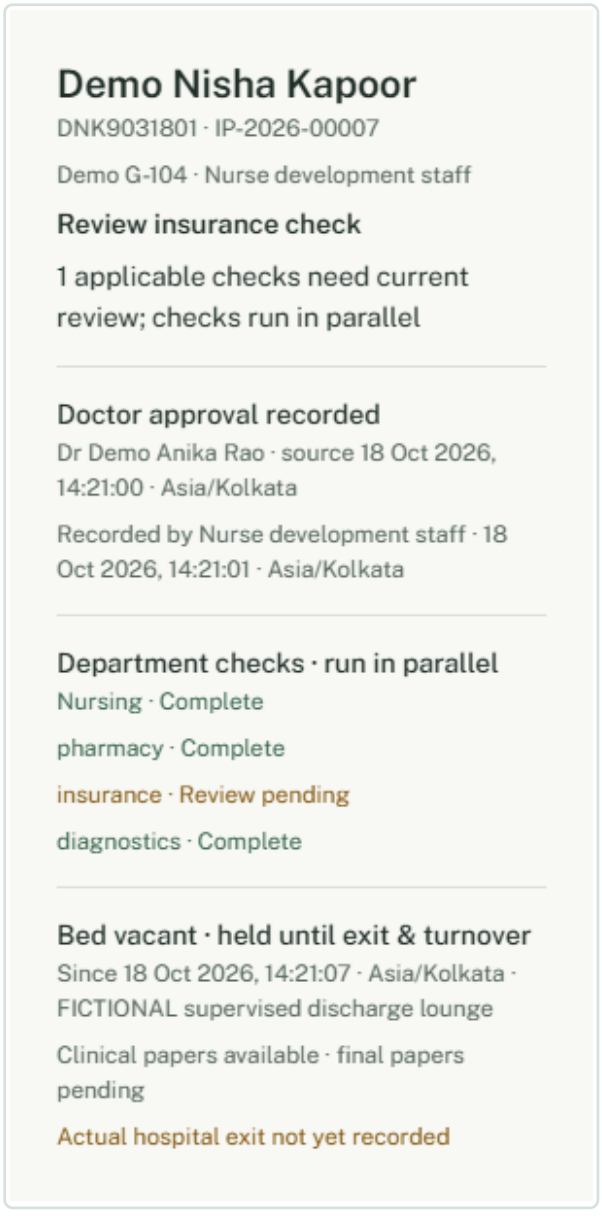
# Vacate the bed and retain waiting care

Vacancy, papers & exit > Record actual bed vacancy

- **Select your recorded care shift** and enter actual vacancy time, the supervised discharge lounge and who continues the waiting-care handover.
- **Click Record physical bed vacancy.** This ends actual room use and posts the configured ₹1,200 room charge. The final net bill is now ₹2,900.
- **The bed remains held.** Nisha has not yet left the hospital. Do not make this room available to another admission just because it is physically vacant.




Record physical vacancy and supervised waiting care




Room use has ended; bed is still held

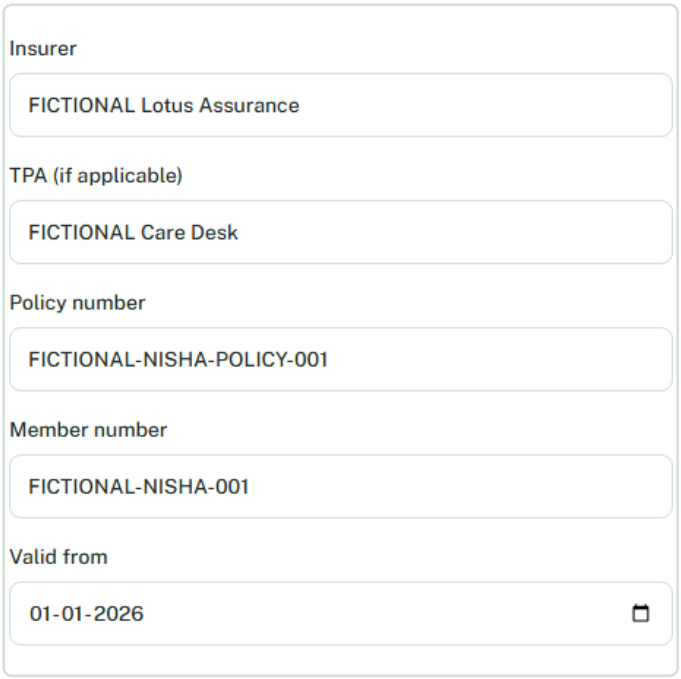
**Expected / verified:** Final bill: room ₹1,200 + consultation ₹800 + blood count ₹400 + medicine ₹600 - return ₹100 = ₹2,900.
**Next:** Insurance verifies responsibility against the final posted bill.

INSURANCE

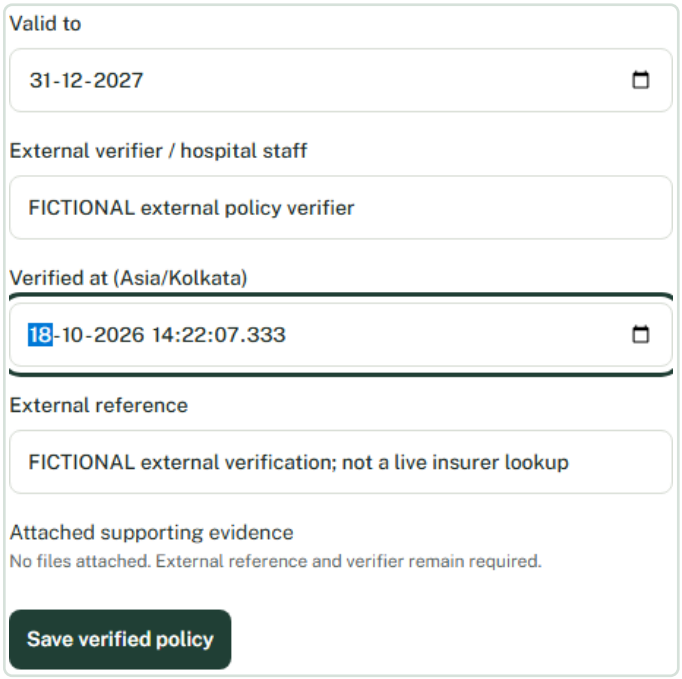
# Enter verified policy details

Insurance cases > Nisha > Decision > Record externally verified policy

- **The case must be assigned to the Insurance user.** Admission created and assigned Nisha's case. The Admission handoff showed a 404; open the saved case from the Insurance dashboard instead (page 21).
- **Enter the fictional insurer/TPA, policy/member numbers and validity dates,** plus external verifier, verification time and reference; click Save verified policy.
- **Verification happens outside the app.** This walk uses fictional references and no supporting uploads. A real case needs its hospital-approved verification and evidence process.




Policy form, upper fields.




Policy form, remaining fields and Save verified policy.

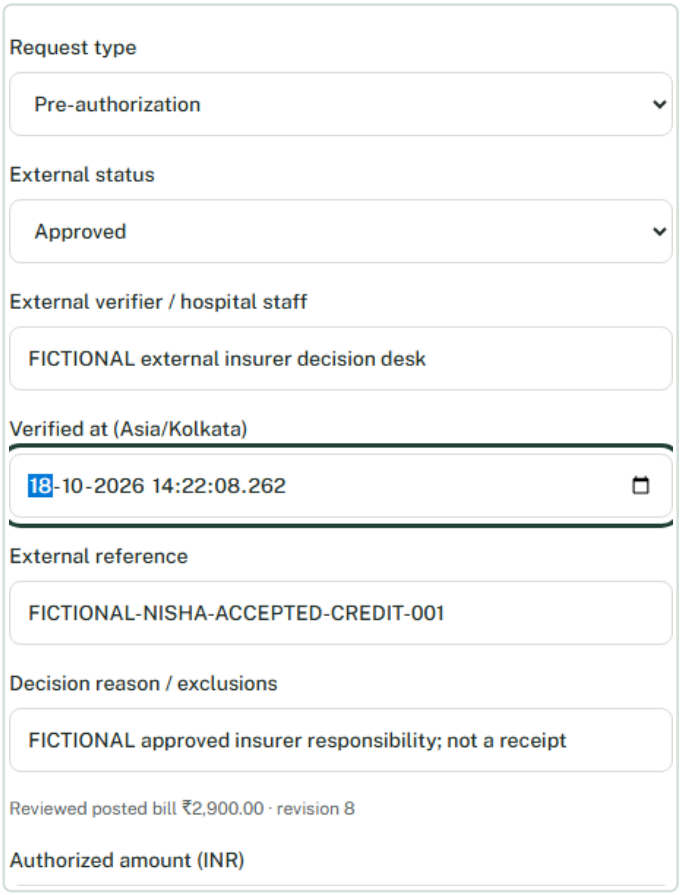
**Expected / verified:** Policy provenance is recorded. This action does not authorize an insurer payment or receive cash.
**Next:** Insurance records the accepted amount and complete party allocation.

INSURANCE

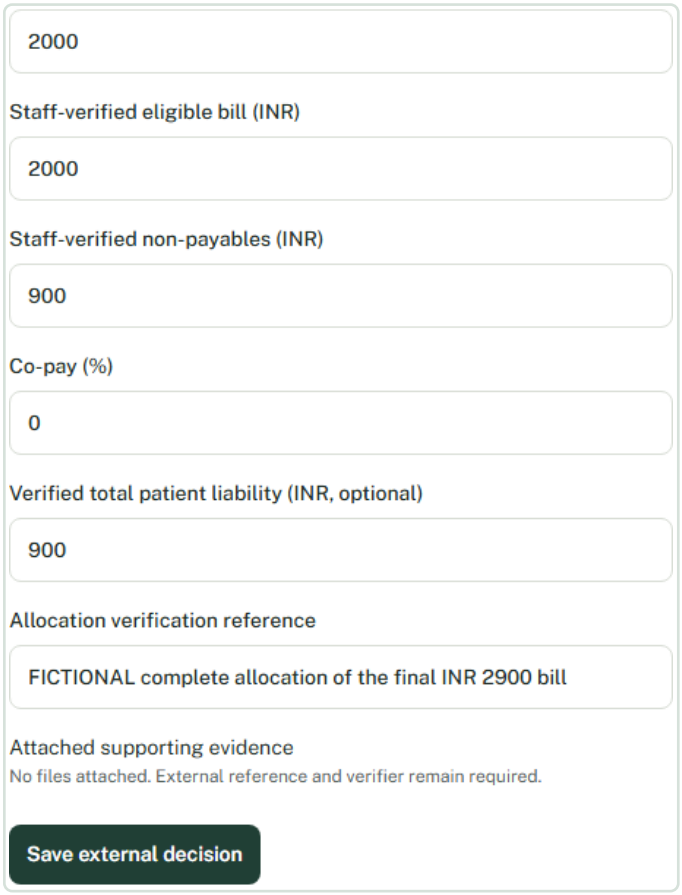
# Record acceptance and party responsibility

Decision > Record payer decision and allocation

- **Choose Approved** and record the external decision reference, verifier, time and exclusions.
- **Allocate the ₹2,900 bill:** authorized insurer amount ₹2,000; eligible bill ₹2,000; non-payables ₹900; co-pay 0%; verified patient liability ₹900. Enter the allocation source reference; click Save external decision.
- **Then open the patient coordination page:** Record your readiness check > INSURANCE > Complete. Record the reviewed responsibility. A changed bill/policy can require a fresh allocation and check.




Decision evidence and reviewed bill binding.




Exact allocation values and Save external decision.

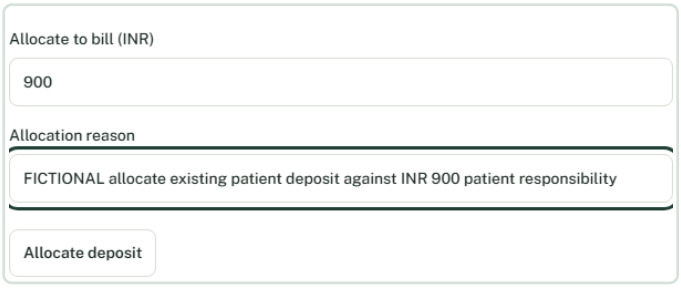
**Expected / verified:** Patient responsibility ₹900; accepted insurer responsibility ₹2,000; actual insurer cash is still ₹0. All four checks are now resolved.
**Next:** Billing settles the patient share and excess deposit.

BILLING

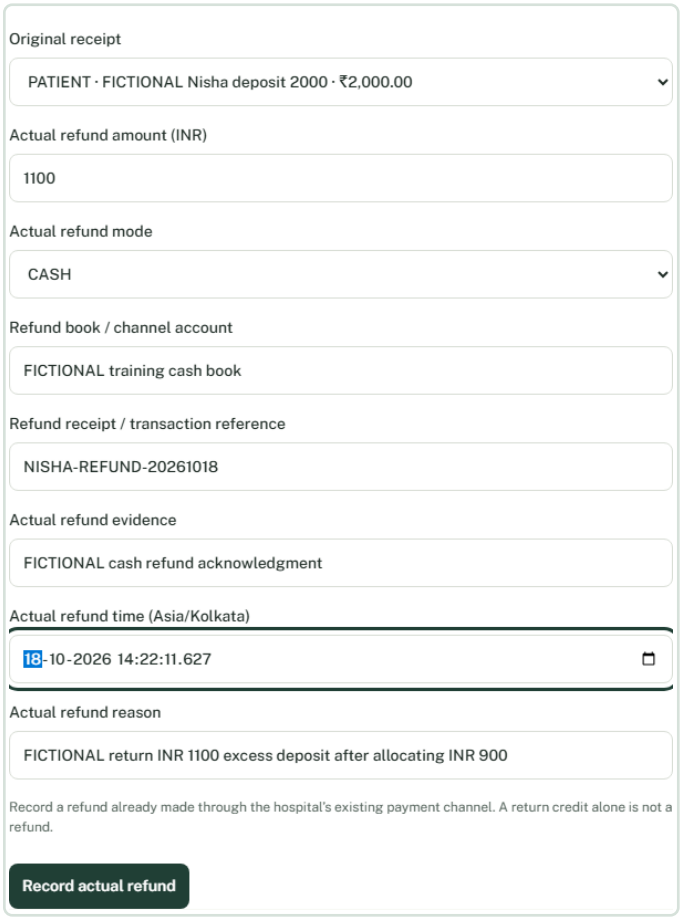
# Allocate the deposit and refund its excess

Money & insurance > Allocate original deposit; Record an actual linked refund

- **Allocate ₹900 from the existing ₹2,000 deposit.** Use Allocate original deposit, the amount and reason, then click Allocate deposit. Allocation creates no new receipt.
- **Record the ₹1,100 excess refund** against that same original receipt, with mode, book, unique refund reference, actual refund evidence/time and reason. Click Record actual refund.
- **Check the numbers:** ₹2,000 received - ₹1,100 refunded = ₹900 retained for the patient share. Do not treat the medicine return credit as a cash refund.




Allocate INR 900 from the existing INR 2,000 deposit




Record the actual fictional INR 1,100 refund

**Expected / verified:** Patient balance is settled and the excess deposit has an attributable refund record.
**Next:** Billing or Admin authorizes final finance.

BILLING / ADMIN

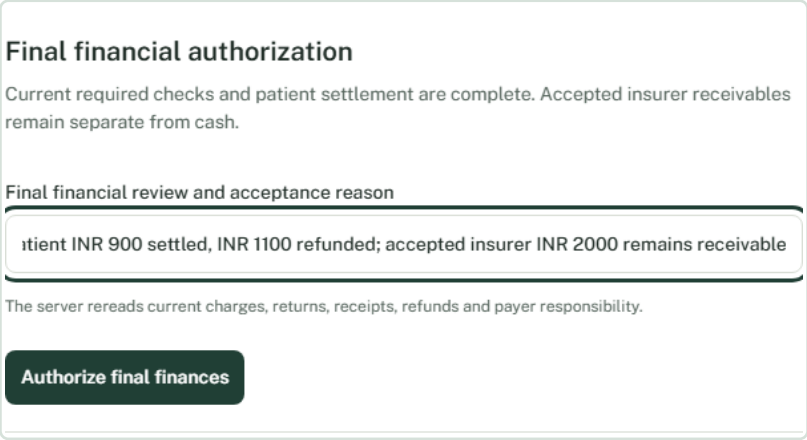
# Review and authorize final finance

Vacancy, papers & exit > Final financial authorization

- **Review the itemized charges and every blocker.** Current checks, room reconciliation, complete payer responsibility, patient settlement and refunds must be resolved.
- **Enter the final acceptance reason** and click Authorize final finances. Either authorized Billing or Admin can do this; it is a separate mandatory action.
- **Retain insurer credit as a receivable.** Nisha's ₹2,000 accepted insurer amount is not cash. Its unpaid status alone does not block this ordinary insured exit after the patient share and required checks are complete.

| Source charge | Quantity / unit | Amount | Status | Ledger correction |
|---|---|---|---|---|
| Demo medicine pack — training only | 6 pack | ₹600.00 | FINAL | ▶ Correct original ledger charge |
| Demo external consultation | 1 visit | ₹800.00 | FINAL | ▶ Correct original ledger charge |
| Demo blood count | 1 test | ₹400.00 | FINAL | ▶ Correct original ledger charge |
| Room · DEMO_GENERAL | 193607 elapsed-ms-cumulative | ₹1,200.00 | FINAL | |

Original charge lines total ₹3,000; the separate accepted return credit reduces the final net bill to ₹2,900.




Authorize final finance: patient settled, insurer receivable retained

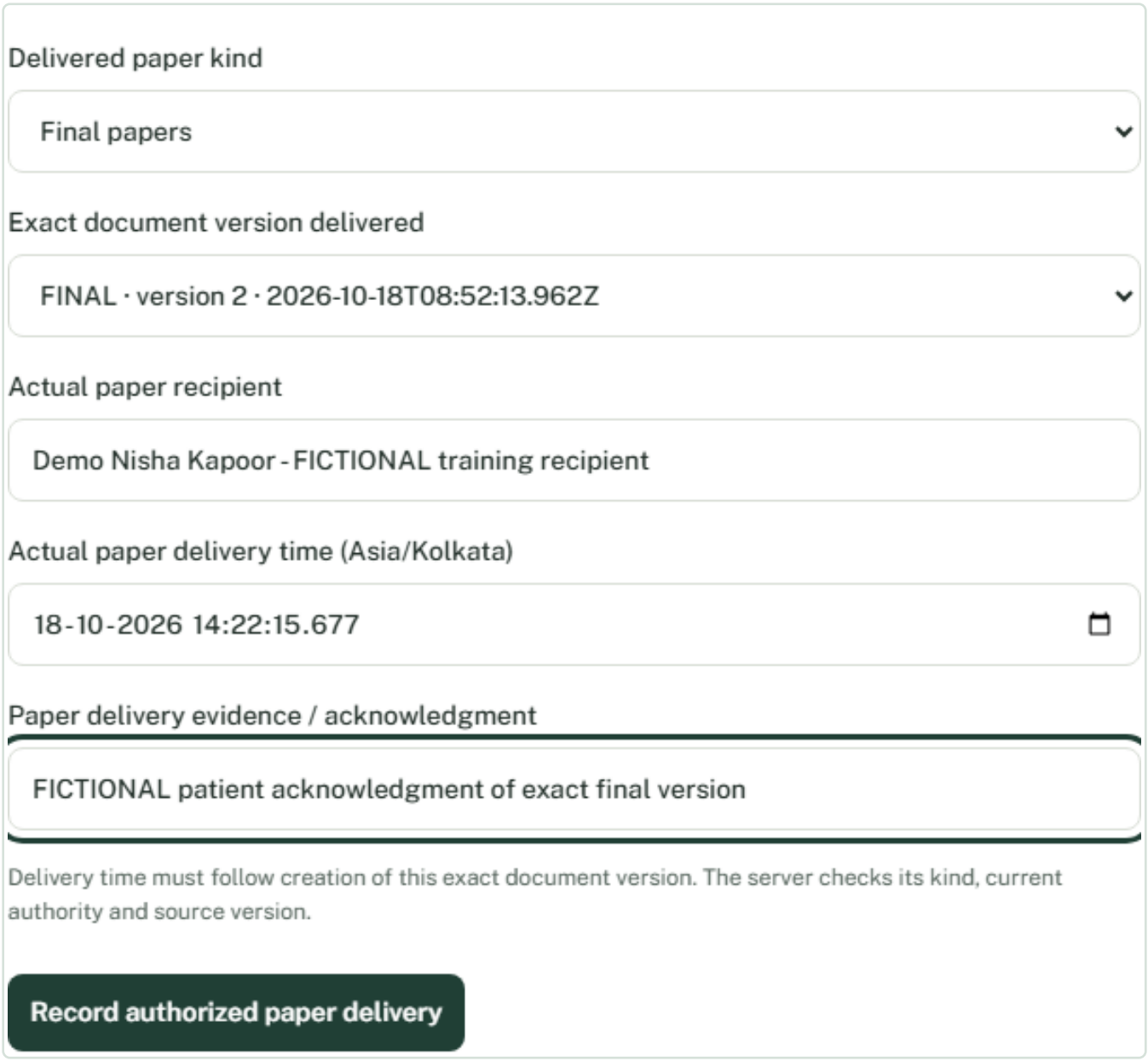
**Expected / verified:** Final finances authorized: net bill ₹2,900; patient retained ₹900; accepted insurer receivable ₹2,000.
**Next:** Nurse generates, reviews and delivers the authorized final papers.

NURSE

# Generate and deliver exact final papers

Vacancy, papers & exit > Generate an authorized paper version > Deliver authorized patient papers

- **Generate FINAL papers after final finance** and open the saved version to review it. A generated file alone is not evidence of delivery.
- **Select FINAL and the exact saved document version,** enter the actual fictional recipient, delivery time after generation and acknowledgment; click Record authorized paper delivery.
- **Keep the distinction:** earlier clinical papers were available after Doctor approval; these final papers rely on final finance. Both versions retain their own history.




Record delivery of the exact final paper version

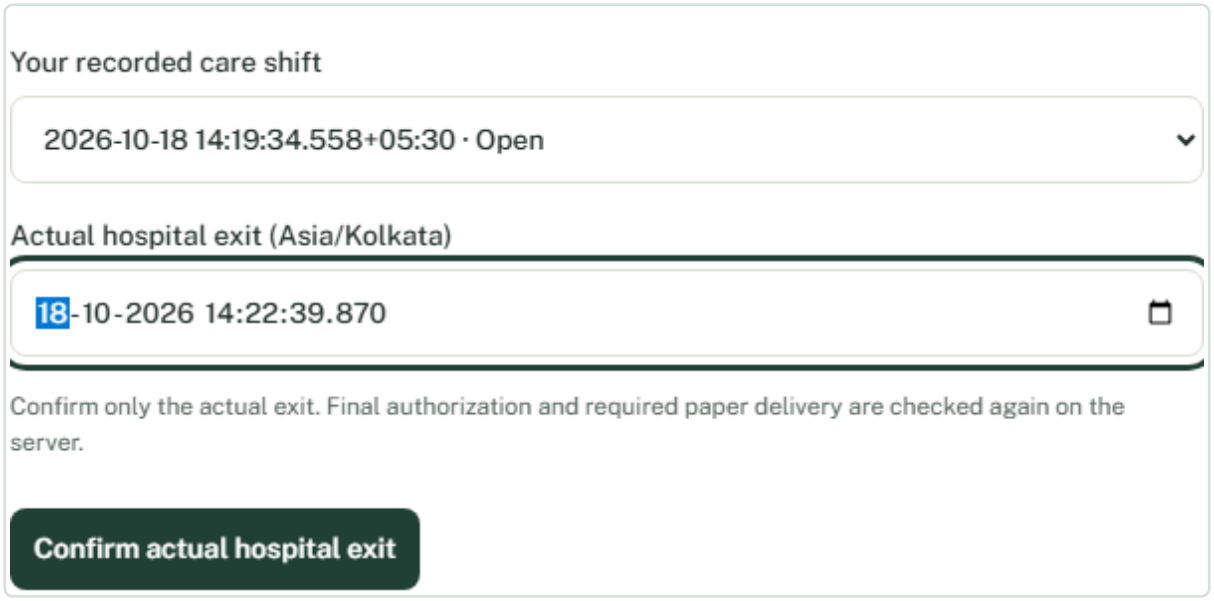
**Expected / verified:** The current final paper version is recorded as delivered before actual hospital exit.
**Next:** The assigned Nurse confirms the patient's actual exit.

NURSE → ADMIN

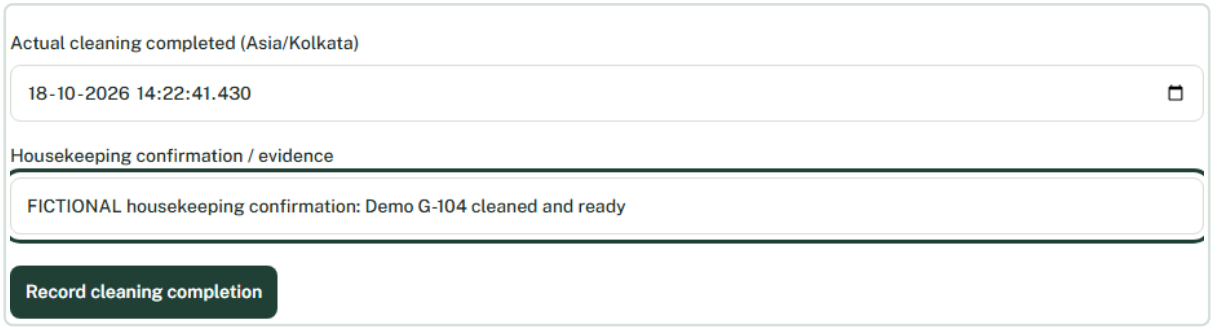
# Confirm exit, then clean the bed

Nurse: Confirm actual hospital exit; Admin: Discharge > Cleaning & turnover

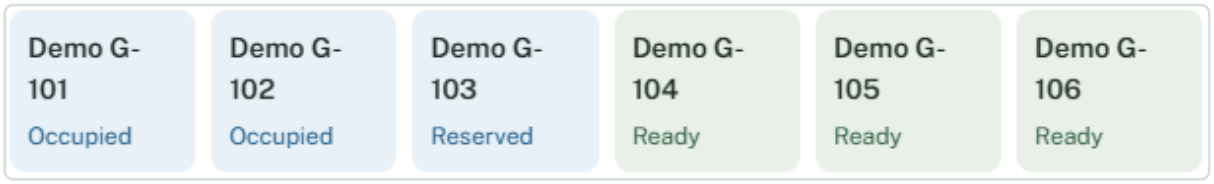
- **Nurse:** choose your care shift, record actual exit after vacancy/finance/delivery and click Confirm actual hospital exit. Nisha becomes DISCHARGED.
- **Admin:** open Discharge and find Demo G-104 under Cleaning & turnover before bed reuse. Enter actual cleaning completion and housekeeping evidence; click Record cleaning completion.
- **Check Beds:** Demo G-104 should now be Ready. Exit releases the patient hold; authorized turnover makes the room reusable. Cleaning controls are on Discharge, not on the Beds card.




Assigned Nurse confirms actual hospital exit




Record authorized housekeeping completion




Demo G-104 is Ready for another patient

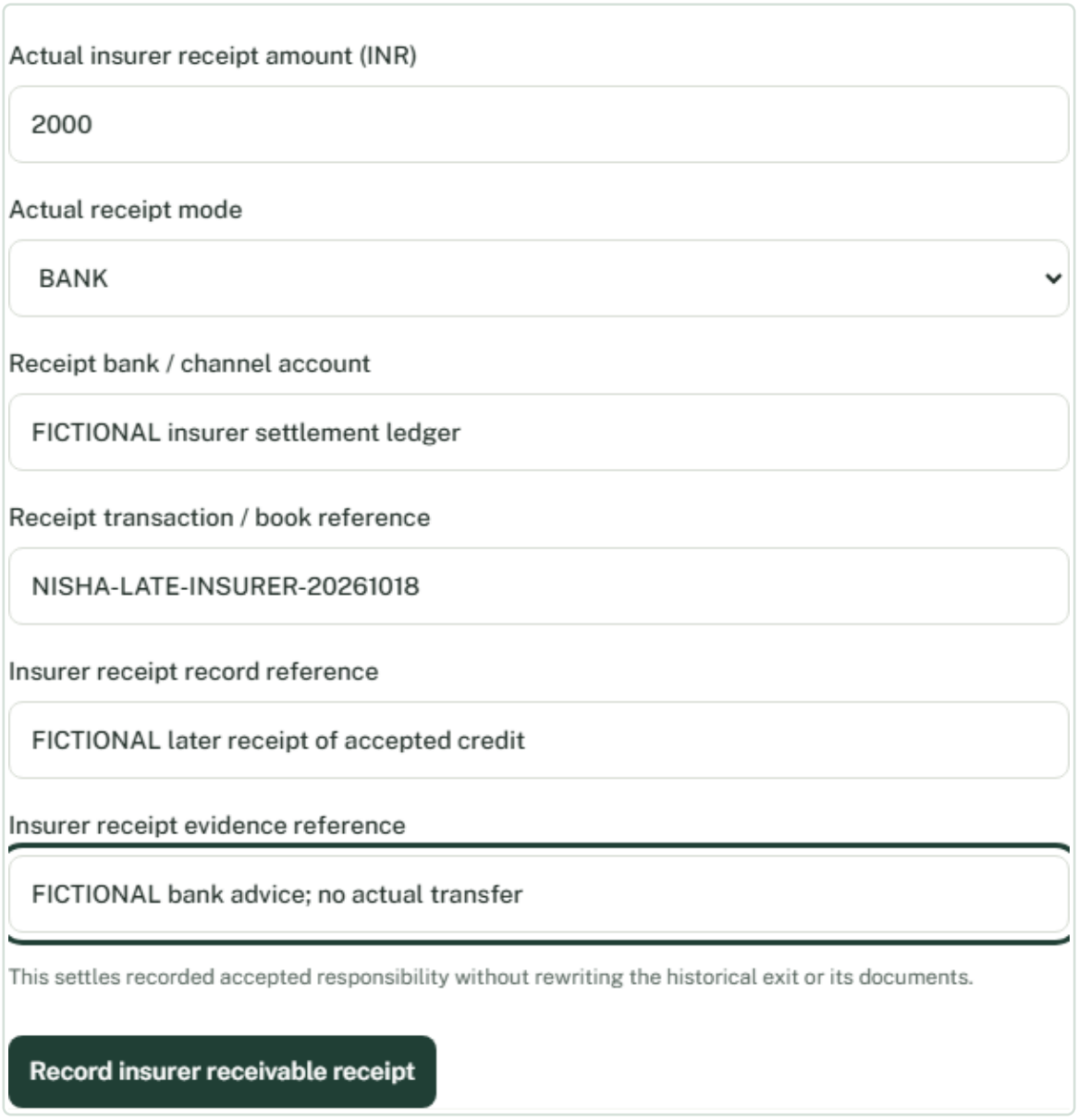
**Expected / verified:** Actual exit is retained and Demo G-104 is clean, free and ready for another patient.
**Next:** Billing can later settle the retained insurer receivable.

BILLING

# Record the later insurer receipt

Retained MVP episodes > Nisha > Money & insurance

- **Open Record actual insurer receipt against receivable.** Enter ₹2,000, BANK, fictional ledger/book reference, unique transaction identity and receipt evidence; click Record insurer receivable receipt.
- **This is a later receipt record,** not a new approval or change to actual exit. In this walkthrough no bank money moved.
- **Verified result:** patient remaining ₹0; insurer remaining ₹0; Nisha stays DISCHARGED. The protected final PDF downloaded before and after this receipt had exactly the same SHA-256 hash.




Record the later fictional INR 2,000 insurer receipt

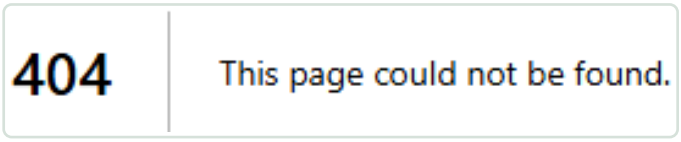
**Expected / verified:** Insurer receivable is settled. Net retained cash ₹900 patient + ₹2,000 insurer = ₹2,900; the original discharge copy is unchanged.
**Next:** Review the observed defect and limits before using this as a staff training guide.

OBSERVED ISSUES

# Two UI issues need attention

Both reproduced during this completed journey

- **1. Admission insurance handoff:** case creation/assignment succeeded, but the admission-linked Insurance detail showed 404 because Reception cannot open the resolved case review. This can make staff think the save failed.
- **Workaround:** return to Admissions and have the assigned Insurance user open Insurance cases. Do not recreate the case. Show a saved-and-assigned confirmation with a role-appropriate handoff instead of 404.
- **2. Misleading closed history:** after the later insurer receipt, the DISCHARGED summary says final papers pending. Exact final-paper delivery is retained and the original final PDF is unchanged. The label is using current finance rather than the pinned exit facts.
- **Workaround and recommended fix:** review the saved final document/delivery and exit history; do not repeat discharge or generate a replacement. Render closed paper/finance status from the original departure authority and recorded delivery.




Issue 1: Admission sees 404 after a successful case save.




Issue 2: the closed summary says final papers pending although delivery and the final PDF are retained.

**Expected / verified:** These are confirmed handoff/display defects. The completed exit, correct money and original PDF were verified. Application code was not changed.
**Next:** Use the next page to distinguish improvements from intentional MVP boundaries.

ATTENTION & REPLAY

# What needs attention next

Observed results, practical improvements and the limits of this one journey

- **Priority 1 - fix the two confirmed UI defects.** Resolve the Admission insurance 404 and misleading closed-paper status on page 21; preserve server permissions, saved case, original finance/delivery and exit history.
- **Usability recommendation:** make the next owner/action and the route back to the same patient more explicit across dashboards. Explain All patients versus Needs action, and show where bed cleaning lives. Reduce repeated time/reference entry while retaining staff confirmation.
- **Manual work is an intentional current boundary:** Doctor decisions, diagnostic confirmation and insurer verification happen externally. No live payer/bank connection, AI/OCR provider or autonomous sender is enabled. No routine Doctor/Lab dashboard or full inventory product is part of this MVP.
- **Before real hospital use:** replace fictional operational settings with approved hospital tariffs/policies, establish actual staff access, backup/restore ownership, secure hosting, document retention and agreed verification/provider processes. Local fictional success does not establish those operating arrangements.
- **Not exercised here:** self-pay, transfers, urgent exceptions, cancellations, unidentified patients, evidence uploads, real insurer/bank communication or automated paperwork. These are not all missing features; they were outside this one ordinary insured walkthrough.
- **How to practise:** create a new fictional insured patient in a Ready bed, use fresh reference numbers and follow pages 2-20. Switch using the profile menu or separate browser profiles. Five sign-ins per minute are allowed; wait a minute if throttled. Passwords are intentionally absent from this PDF.
- **What was checked:** 28 UI action steps across all six roles, 47 captured snapshots, actual database settlement and bed state, preserved earlier rows/credentials/files, and unchanged final PDF bytes after later insurer receipt. Only selected screenshots are reproduced here.
- **Find the completed example:** Retained MVP episodes > search Nisha. Her admission is closed; leave it as history. Existing sample patients and staff accounts were preserved.

**Expected / verified:** One complete fictional insured journey is verified, with two confirmed UI defects and explicitly stated limits.
**Next:** For a new practice case: Admission → Admin assignment → Nurse/Pharmacy/Billing/Insurance → final finance → papers/exit → cleaning → later insurer receipt.